

CLINICAL NOTE – FICTITIOUS PATIENT DATA (FOR TESTING ONLY)

Patient: Gabriela Reyes-Montoya
Date of Birth: 21 July 1989
MRN: 8834712
SSN: 634-28-5510
Address: 217 Sunflower Drive, Houston, TX 77003
Phone: (713) 994-6628
Email: g.reyesmontoya89@outlook.com
Referring Physician: Dr. Carlos Espinoza-Vega, NPI 7723041985
Facility: Memorial Hermann Hospital, Houston

Visit Date: 22 June 2026
Reason for Visit: Breakthrough tonic-clonic seizure – known epilepsy,
query non-compliance vs. breakthrough.

HISTORY OF PRESENT ILLNESS

Gabriela Reyes-Montoya is a 36-year-old female with known juvenile myoclonic epilepsy (JME) presenting via ambulance following a witnessed tonic-clonic seizure lasting approximately 90 seconds at her workplace. She had no aura and was post-ictal for approximately 20 minutes. Her husband Roberto Montoya (contact: 713-445-8812) was contacted and is en route. Her mother Lucia Reyes (home: 713-994-6628) also notified. She is 14 weeks pregnant (G2P1). She had missed two doses of Levetiracetam this week due to nausea.
Last seizure prior to today: 11 March 2024 (18 months seizure-free until today).
Neurologist: Dr. Carlos Espinoza-Vega (clinic: 713-667-2200).

PAST MEDICAL HISTORY

- Juvenile myoclonic epilepsy (diagnosed age 17, on Levetiracetam since 2007)
- Anxiety disorder (GAD, on Sertraline – continued in pregnancy)
- G2P1 – 14 weeks gestation (planned pregnancy)
- Prior febrile seizure (age 3, no recurrence until JME diagnosis)

CURRENT MEDICATIONS

- Levetiracetam 1000mg twice daily (AED – first line for JME in pregnancy)
- Sertraline 50mg once daily (anxiety – continued per psychiatrist advice)
- Folic acid 5mg once daily (pregnancy – high-dose due to AED use)
- Vitamin D 1000 IU once daily

ALLERGIES

Valproate (teratogenic – contraindicated in pregnancy), Penicillin (urticaria)

VITAL SIGNS (POST-ICTAL)

BP: 122/78 mmHg HR: 104 bpm (sinus tachycardia – post-ictal) RR: 16/min
SpO2: 98% on room air Temperature: 37.1 degrees C GCS: 15/15
Weight: 63 kg Height: 164 cm BMI: 23.4 kg/m2

LABS

Hb: 112 g/L (mild anaemia – pregnancy related)

WBC: $11.8 \times 10^9/L$ (mildly elevated – physiological in pregnancy)

Platelets: $288 \times 10^9/L$

Levetiracetam level: 18 umol/L (subtherapeutic – therapeutic range 35-110 umol/L)

Na: 138 mmol/L K: 3.6 mmol/L Glucose: 5.8 mmol/L CRP: 4 mg/L (normal)

Creatinine: 62 umol/L LFTs: Normal Beta-hCG: consistent with 14 weeks

OBSTETRIC ASSESSMENT

Foetal heart rate: 152 bpm (Doppler – normal for gestational age).

No uterine contractions noted.

Obstetrics registrar (Dr. Ana Lima, pager 4421) notified – bedside review requested.

IMAGING

CT head (non-contrast, ALARA protocol, pregnancy shielded):

No intracranial haemorrhage. No space-occupying lesion. No midline shift.

Ventricles normal. No acute abnormality identified.

ASSESSMENT

Breakthrough tonic-clonic seizure in known JME – dose-related (subtherapeutic

Levetiracetam level 18 umol/L, missed doses confirmed).

14 weeks gestation – foetal wellbeing confirmed on Doppler, no obstetric concerns.

Anxiety disorder (GAD) – ongoing, Sertraline continued.

Anaemia of pregnancy – Hb 112 g/L, monitor.

PLAN

1. Oral Levetiracetam dose: take missed doses now (1000mg), resume regular schedule.
2. Uptitrate Levetiracetam to 1250mg twice daily – subtherapeutic level confirmed.
3. Neurology follow-up – Dr. Carlos Espinoza-Vega within 1 week (713-667-2200).
4. Obstetric review – Dr. Ana Lima to assess foetal wellbeing.
5. Driving advice: seizure today resets seizure-free period – must not drive.
6. Seizure first aid education given to Roberto Montoya and Lucia Reyes.
7. Patient ID wristband: 8834712.

Signed: Dr. James Whitfield, Senior ED Registrar

Date: 22 June 2026

Facility Contact: Memorial Hermann Hospital, 6411 Fannin Street, Houston TX 77030